

‘UP YOURS AT THE END OF LIFE’

*Opposition, Emotion, and Overlap in a
Corpus of Scottish Debates on Assisted Dying*

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CILC16, Salamanca
May 2025

LEVERHULME
TRUST _____



University
of Glasgow

CONTENT ADVISORY

- Death (throughout)
- Suicide (throughout)
- Suffering at end of life (throughout)
- Abortion (brief mention in texts)

ASSISTED DYING

- In texts of citizens writing their opinions, for a government audience, on something as controversial as Assisted Dying, what can corpus linguistics bring to the table?
- Three questions:
 - What is linguistically polarized in this debate?
 - How do these polarized ideas lead to contested meanings?
 - What are some societal implications?

OUTLINE

- Background
- Our Corpora
- Polarization and Entrenchment
- Argumentation and Authority
- Implications and Democracy
- Conclusion

1. BACKGROUND

NEWS

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Scottish assisted dying bill passes first vote



| A bill to legalise assisted dying in Scotland has passed an initial vote

Angus Cochrane

BBC Scotland News

13 May 2025

Assisted Dying debate was Scottish Parliament at its best

13TH MAY

[ASSISTED DYING](#)

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By Hannah Brown

Political Correspondent

[@HannahMargBrown](#)

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Liberal Democrat MSP Liam McArthur as MSPs debate the Assisted Dying for Terminally Ill Adults (Scotland) Bill, in the Scottish Parliament in Edinburgh. (Image: Andrew Milligan/PA)

Columnists

The cases both for and against Scotland's Assisted Dying Bill

By Professor June Andrews And Liam McArthur

[Comment](#)

Published 12th May 2025, 18:44 BST

As MSPs prepare to vote on the Assisted Dying Bill, dementia expert Professor June Andrews and Liam McArthur MSP offer arguments from both sides of the debate

When they decide whether to advance legislation on “assisted dying”, MSPs will vote according to their conscience. But their opinions ought to be based on better knowledge about dying, the current law, and a careful scrutiny of what’s missing from this bill.

“Assisted dying” sounds compassionate, even humane. But the claim that there’s public support is built on dangerously simplistic questions, and this is about your conscience, right? Not populism.



SARAH DITUM

Assisted dying can't be sanitised. Stop trying

MPs' attempts to improve Kim Leadbeater's Terminally Ill Adults (End of Life)
Bill have only revealed that the entire project is confused

[Sarah Ditung](#) | Tuesday May 13 2025, 8.43pm BST, The Times

I spent the beautiful early summer weekend sitting on a bench in the sunshine, talking about death. I was with a close friend who works as a hospice chaplain. Much of her job involves dispensing spiritual consolation and makeshift blessings.

But the main thing that troubles the dying, she says, has nothing to do with faith. As people face the end of their life, the same question comes up again and



IAIN MACWHIRTER

I supported voluntary euthanasia before witnessing my mother's death

MSPs vote on Liam McArthur's assisted dying bill on Tuesday but Holyrood has a poor record when it comes to conscience bills like this

[Iain Macwhirter](#) | Sunday May 11 2025, 6.01pm BST, The Times

Reformers are confident that the Liberal Democrat MSP Liam McArthur's Assisted Dying for Terminally Ill Adults (Scotland) Bill will win a majority in the Scottish parliament on Tuesday. I'm not so sure.



Attempts to change the law to allow assisted dying are being made in both the Scottish and UK parliaments (Picture: Leon Neal) | Getty Images



**LET US
CHOOSE**

LEGALISE ASSISTED DYING

Back
**Bodily
Autonomy**



Humanist
Society
Scotland

MY
CHOICE
DIGNITY

GIVE ME **CHOICE**
OVER MY DEATH

**DIGNITY
IN DYING
SCOTLAND**

Back
**Assisted
Dying**

VOTE
YES TO
CHOICE

YES

YES

YES

YES

YES

YES

YES

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Should it be legal for a doctor to assist a terminally ill patient in ending their life by prescribing life-ending medication?

Do you think it should be legal or not for a doctor to assist a patient aged 18 or over in ending their life by prescribing life-ending medication that the patient can take themselves, if all the conditions for assisted dying outlined above are met?*



October 2024



July 2023



Base: 1,130 Online U.K. adults aged 16-75, 1-2 October 2024. * Visit the Ipsos website to see information given to respondents about assisted dying before they answered this question (Slide 2).



PERSONAL EXPERIENCE

“

For 26 years I worked in the emergency service [...] I have witnessed at first hand and close up the sheer desperation and suffering caused by terminal illness. I have had to examine bodies and scenes where the person has died alone and in obvious agony. People with cancer who have vomited and excreted blood and body matter into bowls, coal scuttles, sinks, beds, floors etc.. before finally dying, often over a period of days and weeks [...] I was also called to all suicides and lost count of the number of people I have had to cut down from their nooses whilst their grieving family cried in the adjacent room. [...] In April 2020 I was diagnosed with inoperable pancreatic cancer and given 6-12 months to live if I chose not to have palliative chemotherapy, and possibly 18 months if I chose to undertake it. My world fell apart and I had to break the news to my wife and two sons. [...] I am a proud and private individual who does not want to spend my final days, weeks, or god forbid months, in a hospital or hospice, in pain with no quality of life and having to be lifted off a bed pan or worse! I do not want to be forced into ending my life alone, maybe in my car with the embers of a disposable BBQ smouldering in the boot, or hanging in a lonely wood waiting for some poor unsuspecting member of the public to find me – but with no compassionate alternative that is what I will do.

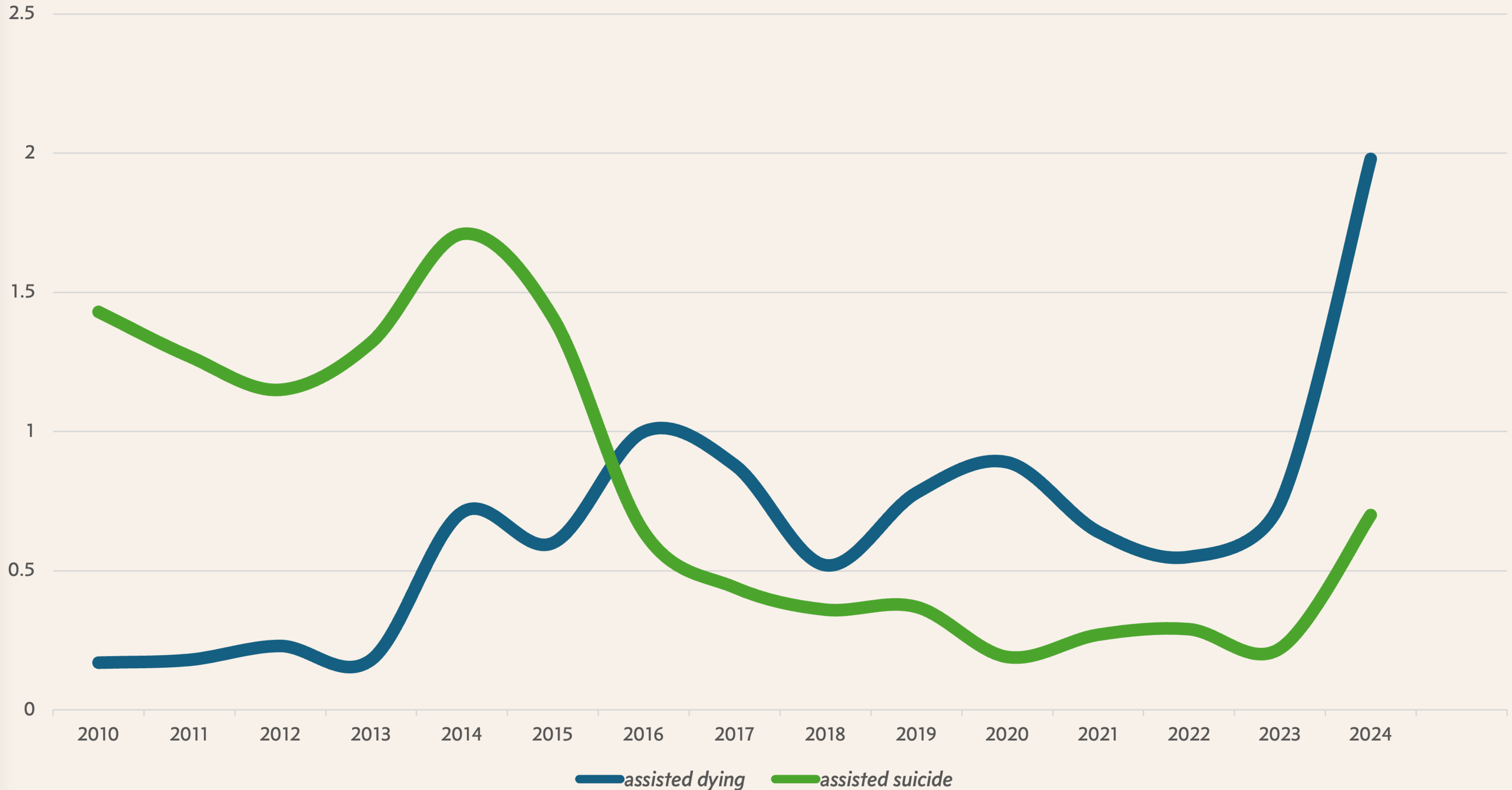
WHY IS THIS INTERESTING?

- A rich dataset (separate to social media) with citizens writing persuasive texts
- Shows profound disagreements between citizens, and is linguistically highly oppositional
- Broad societal implications of outcomes

CURRENT SITUATION

- September 2021: Assisted Dying for Terminally Ill Adults (Scotland) Bill
- Key differences from previous attempts:
 - Limited to terminally ill, mentally competent adults
 - Scottish residency requirement (12+ months)
 - Requires evaluation by two independent doctors
 - Multiple decision points and waiting periods
 - Self-administration requirement

News on the Web (NOW) Corpus, per million words



TERMINOLOGY

No terminology choice is neutral in this debate

- **“Assisted Dying”**: Term used in current bill’s title
- **“Assisted Suicide”**: Primarily used by opponents
- **“Euthanasia”**: Not accurate for current bill (euthanasia is physician-administered and this bill requires self-administration)

2. OUR CORPORA

CORPORA

— Consultation Corpus

- Free text entries from two public consultations
- Metadata includes self-reported closed question on support/opposition to the bill
- 2021: 2m words across 12,255 written responses
- 2024: 1.1m words across 7,122 written responses

— Press Corpus

- Press articles in the UK from the months surrounding the two consultations
- Obtained through a set of seed search terms
- 2021: 2.3m words across 1,668 articles
- 2024: 2.6m words across 2,969 articles

CONSULTATIONS

Two consultation stages:

- **First (2021)**: Conducted by McArthur during bill formulation
 - Largest consultation ever for a Scottish bill (12,255 responses)
- **Second (2024)**: By Health, Social Care and Sport Committee
 - A ‘call for views’ must take place as part of Stage 1 consideration (7,122 responses)

CONSULTATION ONE

Period: 23 September 2021 – 22 December 2021

Individual available responses: 12,255

Word count: 2,018,421

9,651 fully or partially supportive, 1.2m words

2,554 fully or partially opposed, 0.8m words

50 neutral or unsure, 18k words

Ranges from 0 to 9,759 words per response (average 182, median 99)

CONSULTATION Two

Period: 7 June 2024 – 16 August 2024

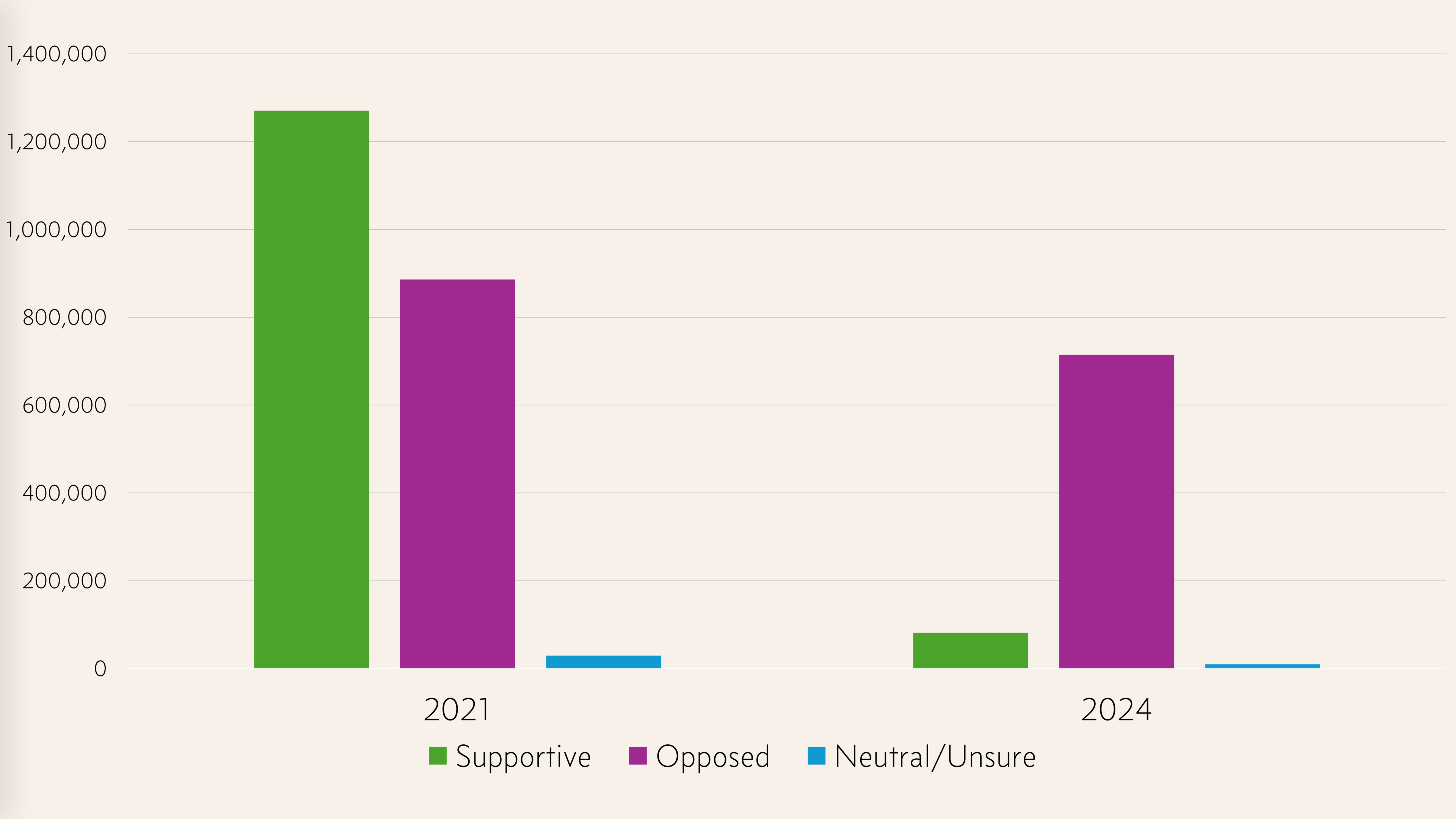
Individual available responses: 7,122

Word count: 805,755

(1,082,534 when including “Other” responses to multiple-choice questions)

425 fully or partially supportive, 81,353 words

6,681 fully or partially opposed, 714,729 words



2021 QUESTIONS

- Question 1: Support for the proposed Bill
- Question 2: Do you think legislation is required
- Question 3: Views of the proposed process
- Question 4: Views on safeguards
- Question 5: Should a body collect data on assisted dying
- Question 6: Views on conscientious objections
- Question 7: Impact on public finances
- Question 8: Impact on protected characteristics
- Question 9: Impact on sustainability
- Question 10: Additional comments or suggestions

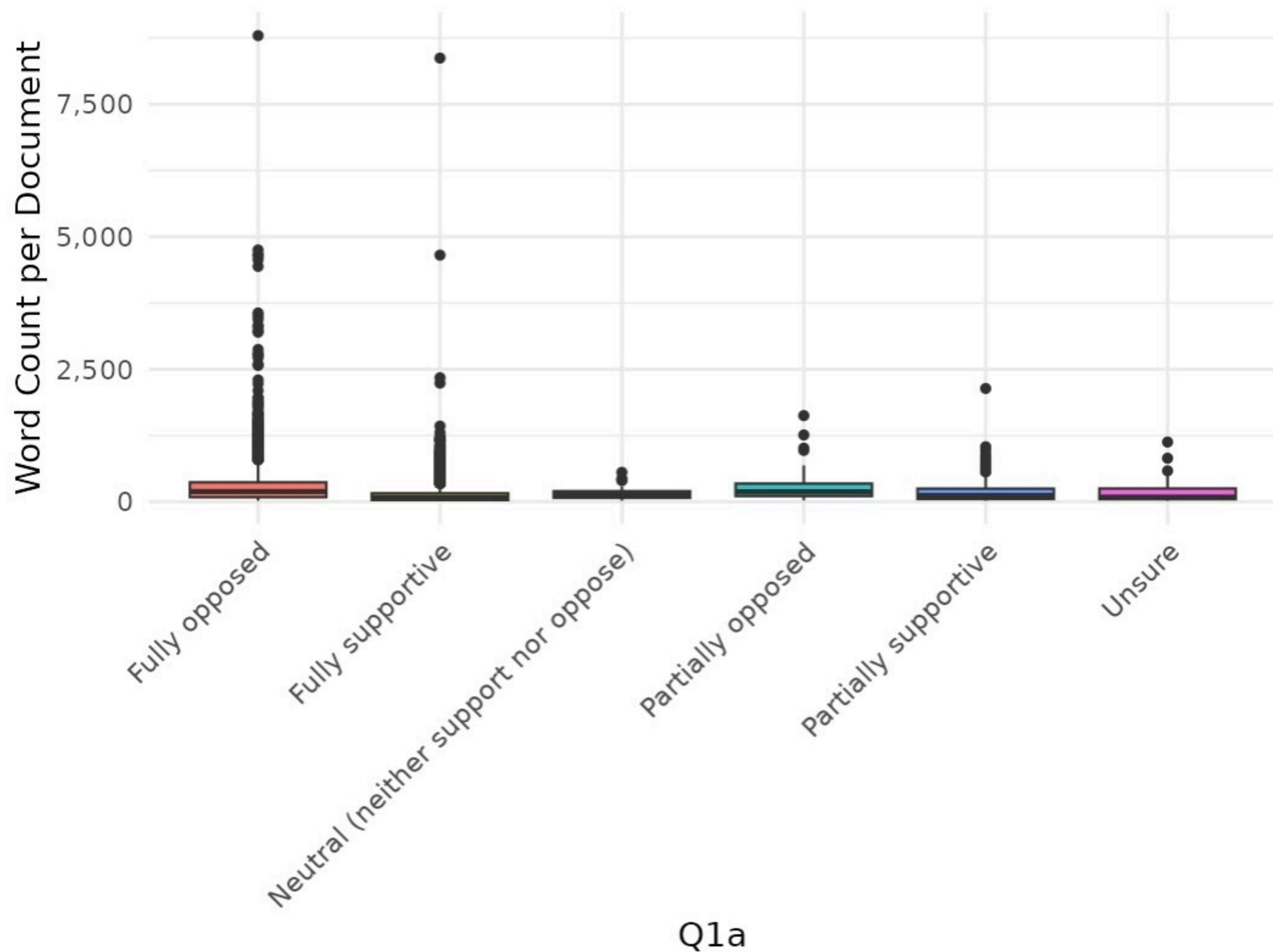
Displaying Document: indiv_163 (Dataset: 2021)

Question 1: Support for the proposed Bill (Choice)

Fully supportive

Question 1: Support for the proposed Bill (Reason)

Millions spent keeping a premature baby alive, up yours at the end of life



3. POLARIZATION AND ENTRENCHMENT

WHAT IS DEATH?

Collocates of *death*:

2021 Supportive			2021 Opposed		
collocation	count	score	collocation	count	score
assisted death	581	3.2	cause death	120	4.38
dignified death	198	4.51	natural death	81	4.67
painful death	190	4.1	death (is) final	45	4.55
bad death	59	4.35	early death	42	4.34
manner death	67	3.69	premature death	41	5.3
good death	86	2.64	choose death	51	3.37
undignified death	37	4.06	listed death	31	5.17
peaceful death	39	3.67	physician-assisted death	29	4.86

WHAT IS DEATH?

Supportive

Death is something to be **judged** and part of **medical care**?

Opposed

Death is something with its own **timescale**, a **threshold** to be respected?

KEY CONCEPTS

- USAS-tagged 2021 corpus (Wmatrix)
- Log-likelihood against BE21 (Paul Baker)

Supportive

USAS

L1-
B2-
E4.1-
L1+
X7+
S6+
Z8
S7.4+
S7.2+
S2
S8+
B3
T2-
Z6
A3+
A9+
S4
Z7
X9.1+
A7+
X2.1
X6+
T1.3++
E2+
S6-

USAS Gloss

Dead
Disease
Sad
Alive
Wanted
Strong obligation or necessity
Pronouns
Allowed
Respected
People
Helping
Medicines and medical treatment
Time: Ending
Negative
Existing
Getting and possession
Kin
If
Able/intelligent
Likely
Thought, belief
Decided
Time period: long
Like
No obligation or necessity

Opposed

USAS

L1-
L1+
S8+
S1.2.5-
B3
I1.3
B2-
S2
T2-
A7+
N3.5
A12-
X2.1
A3+
T3+++
S6+
Z6
N3.8
A15+
S8-
S7.2+
E6-
G2.1+
T1.1.3
E4.1-

USAS Gloss

Dead
Alive
Helping
Weak
Medicines and medical treatment
Money: Cost and price
Disease
People
Time: Ending
Likely
Measurement: Weight
Difficult
Thought, belief
Existing
Time: Old; grown up
Strong obligation or necessity
Negative
Measurement: Speed
Safe
Hindering
Respected
Worry
Lawful
Time: Future
Sad

Supportive

L1- (Dead)
B2- (Disease)
E4.1- (Sad)
L1+ (Alive)
X7+ (Wanted)
S6+ (Strong obligation or necessity)
Z8 (Pronouns)
S7.4+ (Allowed)
S7.2+ (Respected)
S2 (People)
S8+ (Helping)
B3 (Medicines and medical treatment)
T2- (Time: Ending)
Z6 (Negative)
A3+ (Existing)
A9+ (Getting and possession)
S4 (Kin)
Z7 (If)
X9.1+ (Able/intelligent)
A7+ (Likely)
X2.1 (Thought, belief)
X6+ (Decided)
T1.3++ (Time period: long)
E2+ (Like)
S6- (No obligation or necessity)

Opposed

L1- (Dead)
L1+ (Alive)
S8+ (Helping)
S1.2.5- (Weak)
B3 (Medicines and medical treatment)
I1.3 (Money: Cost and price)
B2- (Disease)
S2 (People)
T2- (Time: Ending)
A7+ (Likely)
N3.5 (Measurement: Weight)
A12- (Difficult)
X2.1 (Thought, belief)
A3+ (Existing)
T3+++ (Time: Old; grown up)
S6+ (Strong obligation or necessity)
Z6 (Negative)
N3.8 (Measurement: Speed)
A15+ (Safe)
S8- (Hindering)
S7.2+ (Respected)
E6- (Worry)
G2.1+ (Lawful)
T1.1.3 (Time: Future)
E4.1- (Sad)

Supportive

L1- (Dead)
B2- (Disease)
E4.1- (Sad)
L1+ (Alive)
X7+ (Wanted)
S6+ (Strong obligation or necessity)
Z8 (Pronouns)
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T2- (Time: Ending)
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Opposed

L1- (Dead)
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T3+++ (Time: Old; grown up)
S6+ (Strong obligation or necessity)
Z6 (Negative)
N3.8 (Measurement: Speed)
A15+ (Safe)
S8- (Hindering)
S7.2+ (Respected)
E6- (Worry)
G2.1+ (Lawful)
T1.1.3 (Time: Future)
E4.1- (Sad)

Q1 USAS ('KEY CONCEPTS') AGAINST BE21

—Supportive top concepts:

- L1- (Dead), B2- (Disease), E4.1- (Sad), L1+ (Alive), X7+ (Wanted)
- S6+ (Strong obligation or necessity), Z8 (Pronouns), S7.4+ (Allowed)
- S7.2+ (Respected), S2 (People), S8+ (Helping)

—Opposed top concepts:

- L1- (Dead), L1+ (Alive), S8+ (Helping), S1.2.5- (Weak)
- B3 (Medicines and medical treatment), I1.3 (Money: Cost and price)
- B2- (Disease), S2 (People), T2- (Time: Ending), A7+ (Likely)

2021 “EASY READ CONSULTATION SUMMARY”

“Over three quarters supportive”

“Lots of these people thought:

- it would be kind to let adults who are terminally ill have a safe assisted death if they wanted it.
- it would stop their pain and suffering.
- assisted dying should be allowed in Scotland because it’s allowed in some other countries.”

PERSONAL EXPERIENCE: SUPPORTING

“It’s about choice. Currently, other people’s choices prevent those who are terminally ill from dying when they wish to. That’s cruel and unfair.”

“It’s about compassion and autonomy. I have medical knowledge and know that some natural deaths can never be made ‘good’. Drugs have their limits.”

“We will surely look back and wonder why it took so very long and why so many people had to yield to a process which denies them control, autonomy and dignity, because we were not brave enough to approach this subject in a calm, rational and compassionate way.”

PERSONAL EXPERIENCE: SUPPORTING

“I had to watch my dad die of pancreatic cancer.”

“I was fully prepared to put myself at risk and buy him drugs from the dark web so that he could stop the pain. However, my daddy told me it was too risky for me, as I work in the legal sector. So I didn’t take it any further. But in what world should this be allowed to happen?”

“I’m sitting here in tears thinking about him. I just wish I hadn’t listened to him and got him the drugs. Even in the hospice he was still saying if there was an injection or a pill, he would take it. Nobody should need to go through this. It’s killing me. He should have been able to leave on his own terms. He shouldn’t have had to suffer and waste away. We don’t treat dogs like this.”

EXCERPTS

2021 “EASY READ CONSULTATION SUMMARY”

“JUST UNDER A QUARTER NOT SUPPORTIVE”

“

“Some people thought:

- it was wrong for a person to take their own life, or to get help to end their life.
- it wouldn't be safe because you wouldn't know for sure if the person was making the choice for themselves.
- older people or disabled people might feel worried about other people trying to choose for them. They might feel less important.”

RELIGIOUS PERSPECTIVE: OPPOSING

- “This is a radical change attacking the very roots of welfare and wellbeing – and trust – and would change society.”
- “We were created in the Image of God – and if this goes forward, we are challenging LIFE – and VALUES and STANDARDS. Does LIFE mean we have a new meaning of LIFE? Parts of society decided to take life in the womb before life was born into the world – now some want to end life before life is over.”
- “One day each of us has to stand before Almighty God – and answer for the decisions we have made – whether we believe it or not – whether we want to or not.”

“SLIPPERY SLOPE” CONCERNS

- “Thus Belgium’s initial law in 2002 expanded from almost exclusively terminally ill cancer patients to patients with psychological illnesses such as severe depression or personality disorder and more recently to competent minors. In Canada, the 2016 law which allowed medical assisted dying for physical health reasons was expanded in 2021 to include patients with mental health conditions.”
- “In Oregon, 5% of patients dying by assisted suicide gave financial reasons for their decision. Those giving as the reason the ‘becoming a burden on family, friends or caregivers’ increased from 13% in 1998 to 55% in 2017.”
- “The British Geriatrics Society, the Association for Palliative Medicine of Great Britain and Ireland, the majority of the British Medical Association members who are involved in palliative care or geriatric medicine oppose the legalisation of assisted dying.”

4. ARGUMENTATION AND AUTHORITY

UNIQUE LEXIS IN SUPPORTIVE SUBCORPUS

suffering

—abject, agonies, brutal, crying, hideous, starve, tears, thirst, torment, torture, traumatising, tumours

humiliation

—bladder, cleaned, drip, eating, humiliation, incontinence, limbs, nappy, urine, vegetable, wash, wearing

prolonging

—drag, dragged, endless, endlessly, lasted, linger

UNIQUE LEXIS IN SUPPORTIVE SUBCORPUS

- “I have seen first hand the unnecessary pain and torment that is caused by this horrific disease.”
- “If you’d seen your Parent with advanced Alzheimer’s, needing to be fed, washed, nappy changed, YOU WOULDN’T NEED TO ASK!”

“WATCH* MY”

positive for Huntington's Disease I've		watched my		3 older sib
I		watched my		dad die sl
I sadly		watched my		young brot
position More than you know		Watching my		lovely str
I		watched my		father die
I		watched my		mother die
I		watched my		daughter c
When I was 19 I		watched my		mother was
I		watch my		father die
I had to		watch my		mother suf
I		watched my		mum suffer
before the experience we had		watching my		dad suffer
I		watched my		husband in

“FIRST HAND”

pain endured I have watched	first hand	my granny suffer g
adult life I have witnessed	first hand	many deaths
and MND and have seen	first hand	their suffer
should end I have seen	first hand	my Father
living is I have had	first hand	experience
living I have witnessed at	first hand	the pain a
family not machines I have	first hand	experience
consultation document I have seen	first hand	a relative
so I have seen at	first hand	the physic

UNIQUE LEXIS IN OPPOSED SUBCORPUS

bill as slippery slope

—eroded, implies, message, normalisation, normalise, opens, promoting, reinforce, sends, signal, subtle, wedge, widen, widened

value of human life

—burden, burdensome, contradiction, devalue, undermine, undermines

appealing to authority

—American, BMJ, cited, commandments, God's, journal, Mr, professor, shalt, thou

UNIQUE LEXIS IN OPPOSED SUBCORPUS

“The Bill would send a signal that some lives are no longer worth living.”

“We are also disobeying one of the 10 commandments which states ‘thou shalt not kill’.”

“CLEAR MESSAGE”

	society	This bill sends a		clear message		that
		and it would send a		clear message		to our
	Assisted suicide sends a		clear message		to fr	
suffering	Assisted suicide sends a		clear message		to fr	
	suffering It also sends a		clear message		to fr	
	Assisted suicide sends a		clear message		to fr	
suffering	Assisted suicide sends a		clear message		to fr	
suffering	Assisted suicide sends a		clear message		to fr	
	Assisted suicide sends a		clear message		to fr	
	Assisted suicide sends a		clear message		to fr	

COMPARATIVE USAS, LOG RATIO

Supportive vs Opposed:

- L2 (Living creatures: animals) - Dogs are ‘treated better’
- N3.8- (Speed: Slow) - Prolonged death
- B4 (Cleaning and personal care) - Personal narratives of humiliation

Opposed vs Supportive:

- N3.8 (Measurement: Speed) - Slippery slope re: rates
- N3.5 (Measurement: Weight) - Pressure on elderly/sick
- I1.3 (Money: Cost and price) - Danger of ‘saving money’
- Z3 (Other proper names) - Appeal to authority

OVERALL

Supportive	Opposed
animals	slippery slope
prolonged death	danger of ‘saving money’
personal narratives of suffering and humiliation	appeals to authority
logical argumentation	too soon
experiential-based	principle-based

5. IMPLICATIONS AND DEMOCRACY

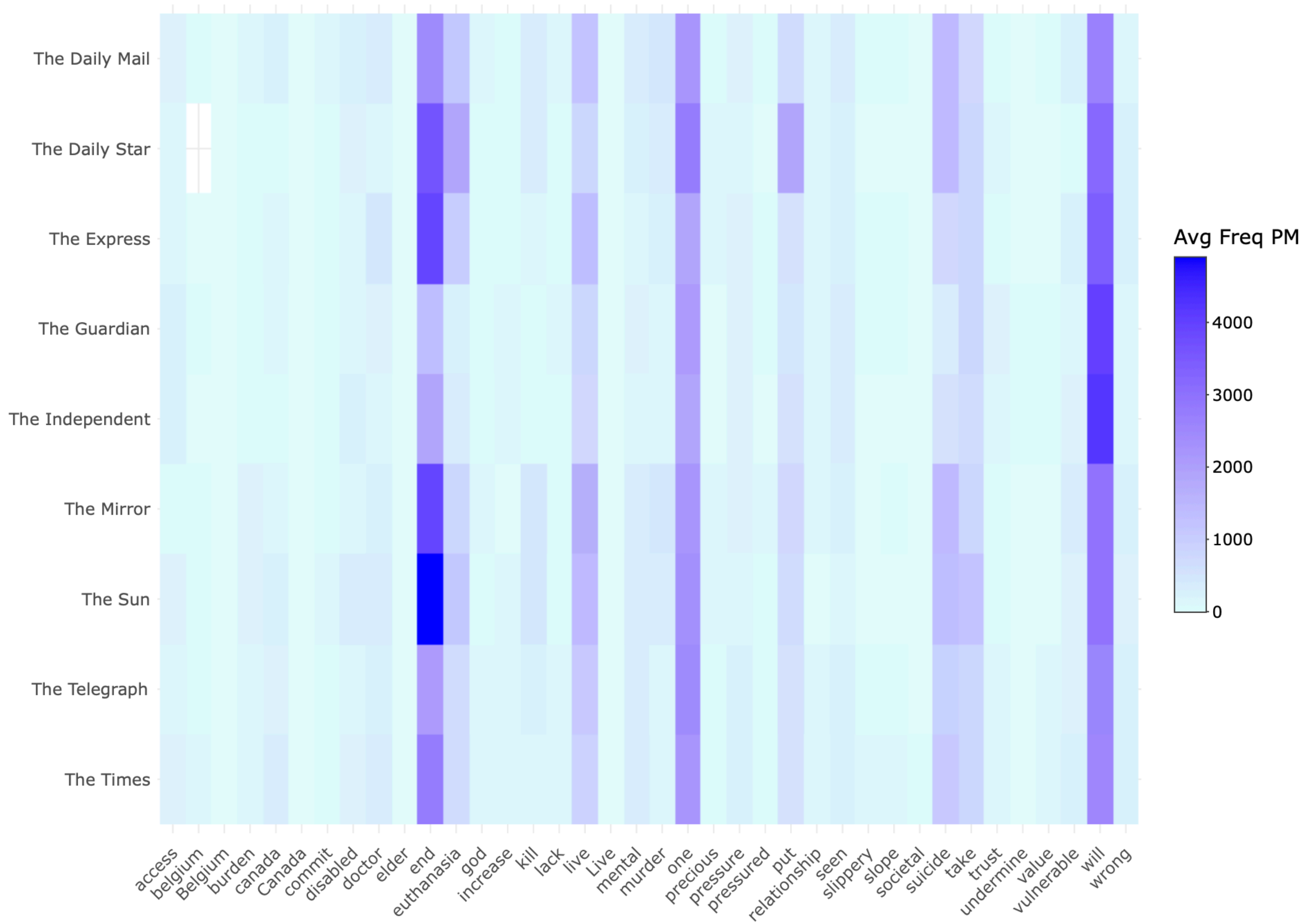
Displaying Document: indiv_163 (Dataset: 2021)

Question 1: Support for the proposed Bill (Choice)

Fully supportive

Question 1: Support for the proposed Bill (Reason)

Millions spent keeping a premature baby alive, up yours at the end of life



MAIL ON SUNDAY (26 DECEMBER 2023)

“ CRISPIN COLEGATE, Pontardawe, Neath Port Talbot. I watched my mother-in-law slowly dying for nearly a year, kept alive by medicines and morphine. She asked me to help her die and I nearly did. Looking back, I wish I had helped her, it would have been worth a prison sentence.

“ KEITH McCREADY, Bishop's Stortford, Herts. IF I had let any of my pets suffer as Sarah Vine's grandmother did in her last days (Mail), I would be prosecuted for cruelty.

MIRROR (23 OCTOBER 2021)

“ Critics fear it could leave vulnerable people exposed to unwanted pressure. Opponents include the Archbishop of Canterbury, the Most Rev Justin Welby. He said that while the safeguards in the bill are stronger than in previous attempts, they still do not go far enough. “What we want is assisted living, not assisted dying,” he said.

POLITICAL OPINIONS

Traditionally:

“Left Wing”

Collective protection of the weak and vulnerable in society

Progressive values

“Right Wing”

Individual responsibility and personal freedom

Conservative values

POLITICAL OPINIONS

But:

- Discourse **opposing** AD is about collective protection of the weak
- Discourse **supporting** AD is about personal responsibility and freedom

MOST FREQUENT SHARED SENTENCES

Shared Sentence Text	Docs	Supportive	Opposed	Neutral
no safeguards are good enough.	26	0	26	0
creating a body responsible for reporting and collecting data can not address possible abuse.	25	0	24	1
terminal illness diagnoses can be unreliable.	24	0	23	1
this is coercive and has no place in a civilised society.	23	0	22	1
in the absence of the bill, such pressure would not exist.	22	0	22	0
life is precious.	21	0	21	0
life is sacred.	21	0	21	0

COPY-PASTE EVIDENCE

Equalities

Q8: We suggest selecting **'Negative'**.

Please use the box to say that the proposals would have a negative impact on the elderly and disabled, to whom the Bill would send a signal that certain lives are no longer worth living. It is also individuals in these categories who are most likely to feel pressurised into considering assisted dying. In the absence of the Bill, such pressure would not exist.

Suggested answer wording

Two responses (3,758 and 3,991 words) with near-identical text



CARE

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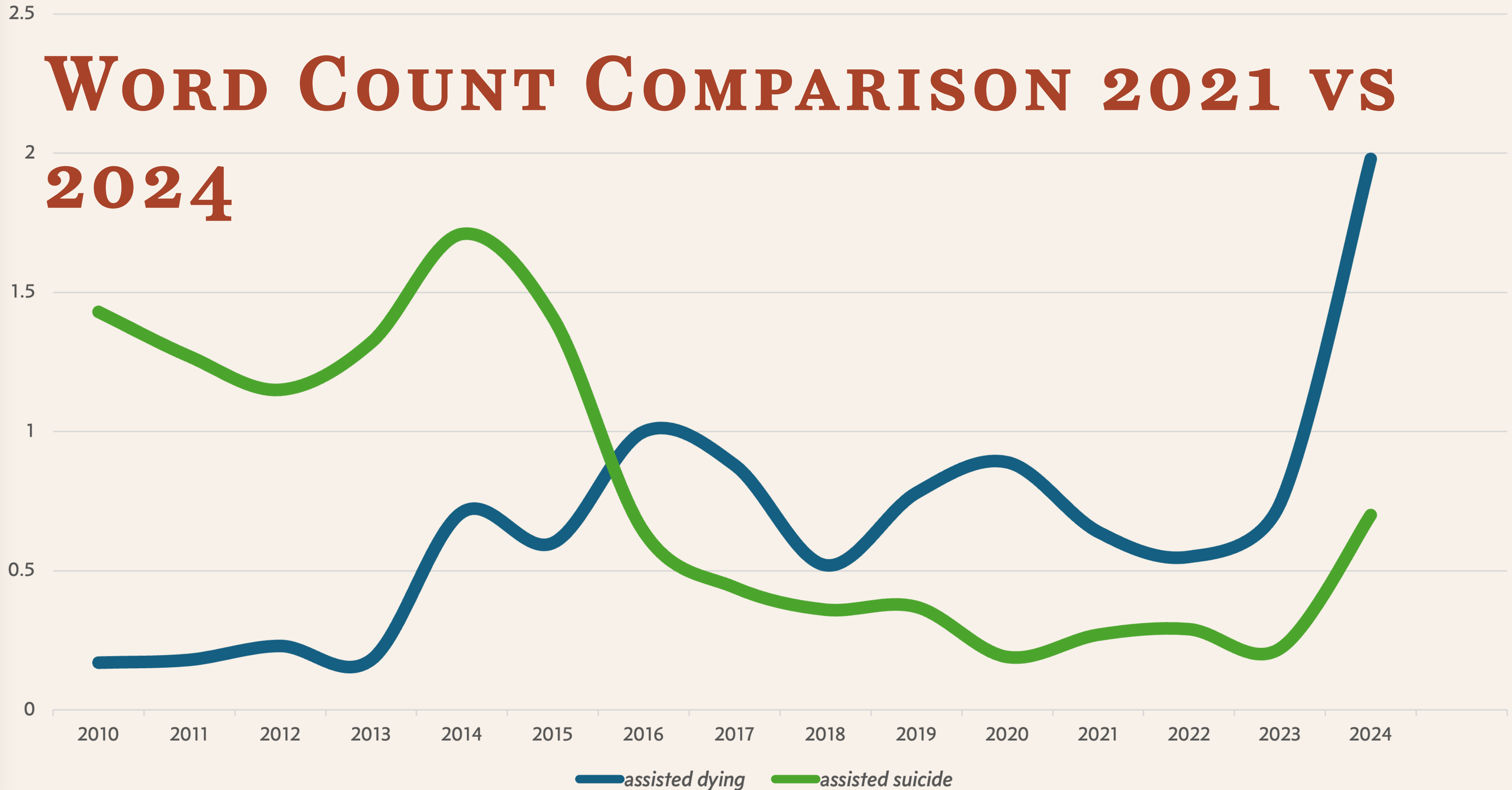
Consultation on Assisted Dying For Terminally Ill Adults (Scotland) Bill - Guide

"Scotland has a proud heritage as a caring nation. All that is being undermined by this new Bill. I'd like to invite you to stand with us as we say 'no' to assisted suicide and 'yes' to better quality end of life care."

Dr Stuart Weir

Source campaign guide

WORD COUNT COMPARISON 2021 VS 2024



6. CONCLUSION

OUR QUESTIONS

We began with three questions:

1. What is linguistically polarized in this debate?
2. How do these polarized ideas lead to contested meanings?
3. What are some societal implications?

ELUSIVE CONSENSUS

These linguistic polarisations show societal and ethical divisions. The use of a fundamentally different ethical language means three things are contested in these debates:

- **Contested conceptualization of death:** is death conceptualized as part of medical care (to be optimised) or as a moral threshold (to be respected)?
- **Contested linguistic authority:** experiential-based versus principle-based language: “I watched my mother die” versus “life is always sacred”/“doctors should never kill”; choice/dignity/decide versus vulnerable/pressure/safeguards; crafting individual moral reasoning versus amplifying that of others.
- **Contested meaning of the vulnerable:** those facing painful death or those facing pressure to end their own lives?

“UP YOURS AT THE END OF LIFE”

OPPOSITION, EMOTION, AND OVERLAP IN A CORPUS OF SCOTTISH DEBATES ON ASSISTED DYING

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CILC16, Salamanca

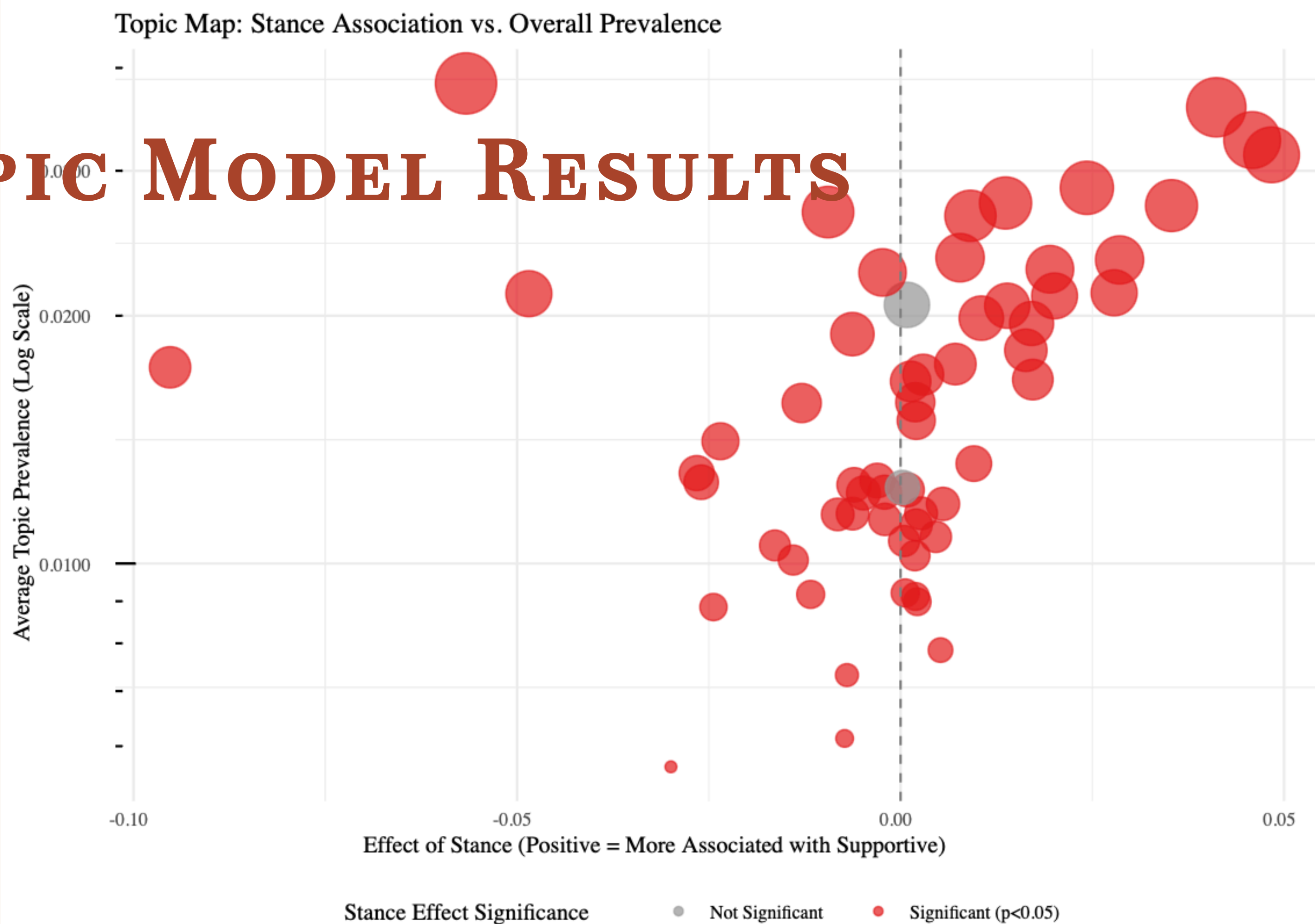
May 2025

APPENDIX

TOPIC MODELLING

- Structural Topic Model (STM) fitted to the combined free-text responses from the 2021 and 2024 consultations
- Respondents classified as ‘Supportive’ or ‘Opposed’ based on their answers to initial multiple-choice question
- Preprocessing (lowercasing, removing punctuation/numbers/stopwords, stemming)
- Modelled using STM to identify 60 distinct topics

TOPIC MODEL RESULTS



Labels show Topic number for top 15% by prevalence or effect size (if toggled on).
Point size reflects prevalence. Red points indicate significant stance effect.

TOPIC 8

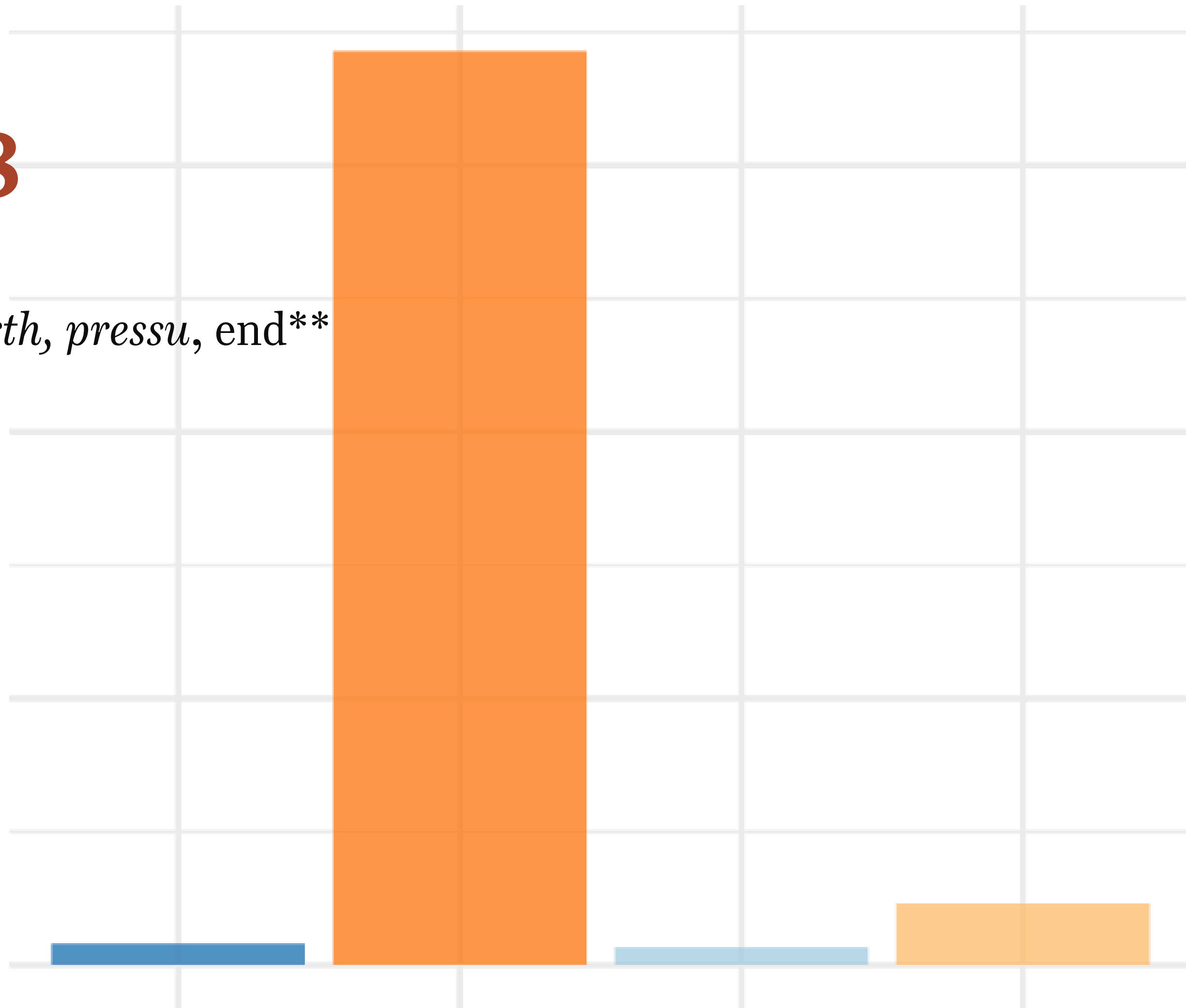
*8: live, elder, worth, pressu, end***

0.075

0.050

0.025

0.000



TOPIC 32

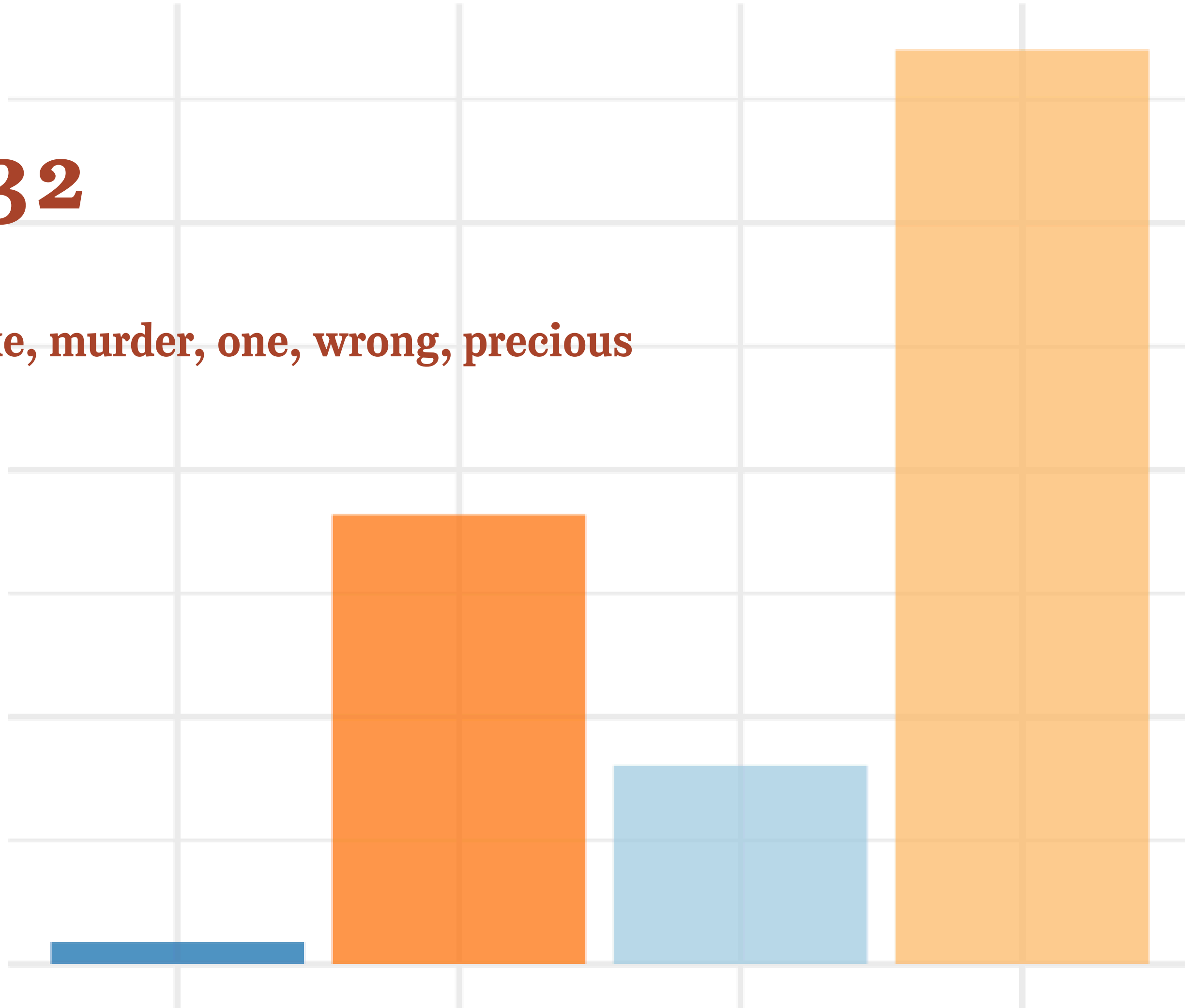
32: kill, god, take, murder, one, wrong, precious

0.09

0.06

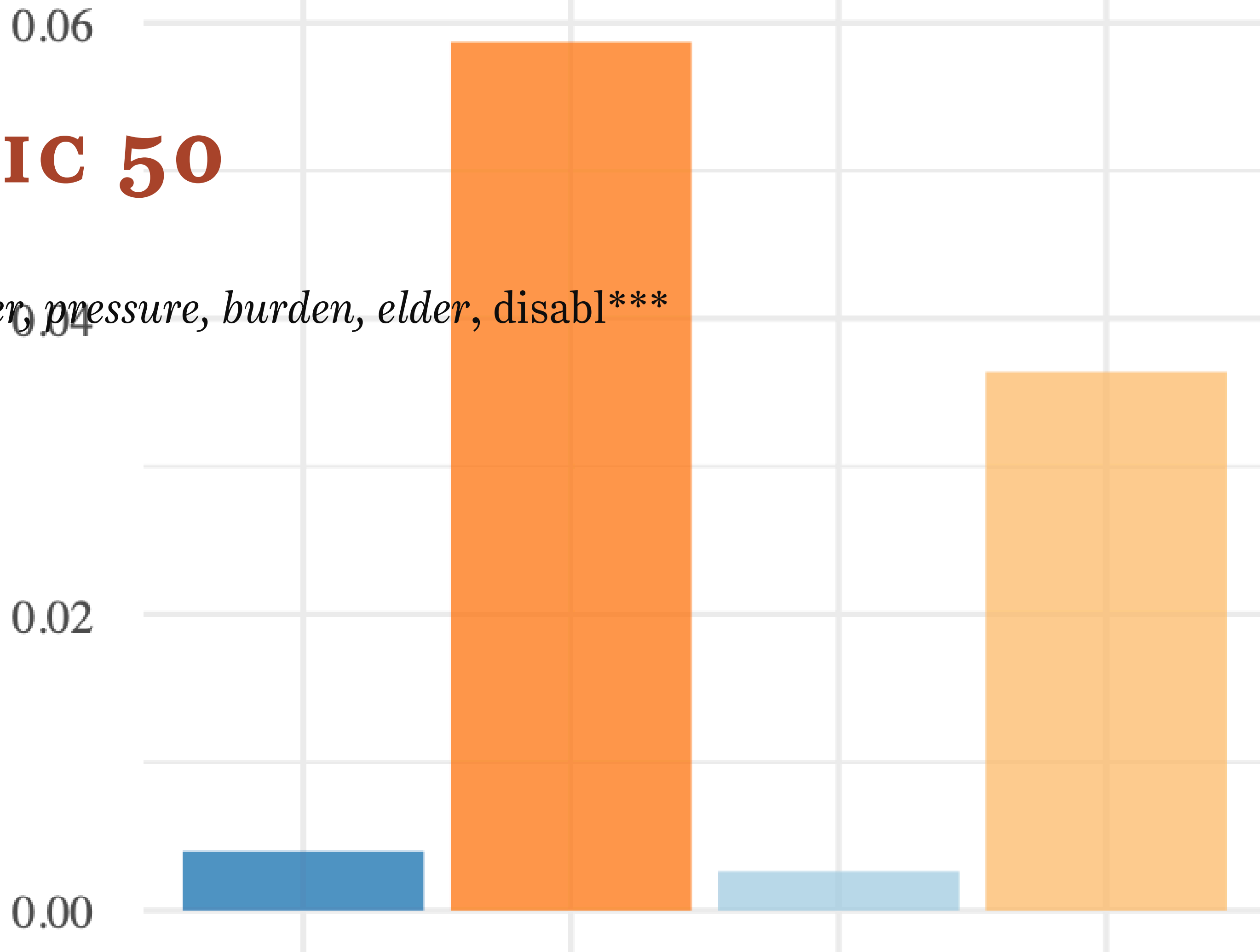
0.03

0.00



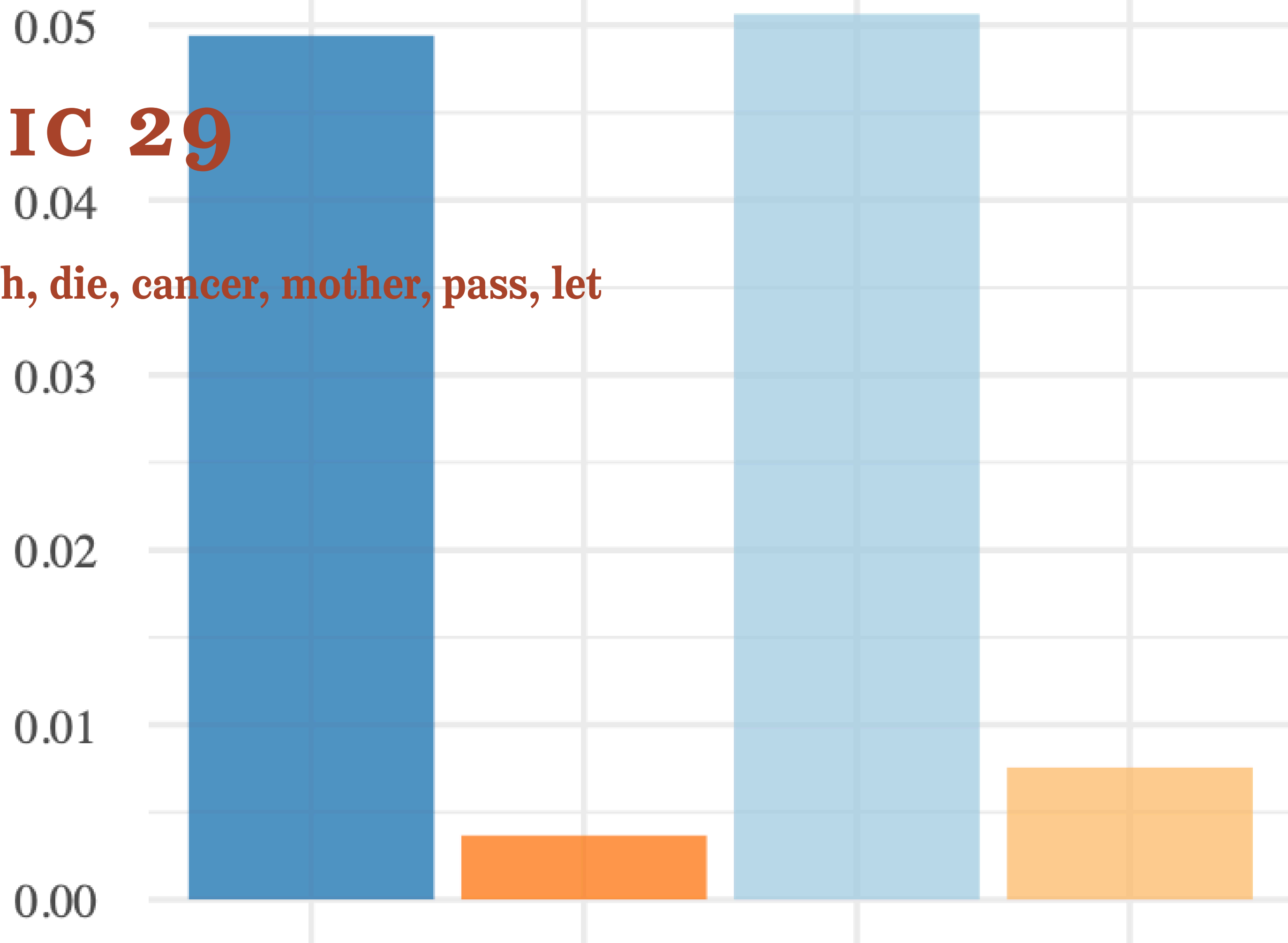
TOPIC 50

*50: vulner, pressure, burden, elder, disabl****



TOPIC 29

29: watch, die, cancer, mother, pass, let



TOPIC 40

40: choice, right, choose, dignity, person, dignif*

0.05

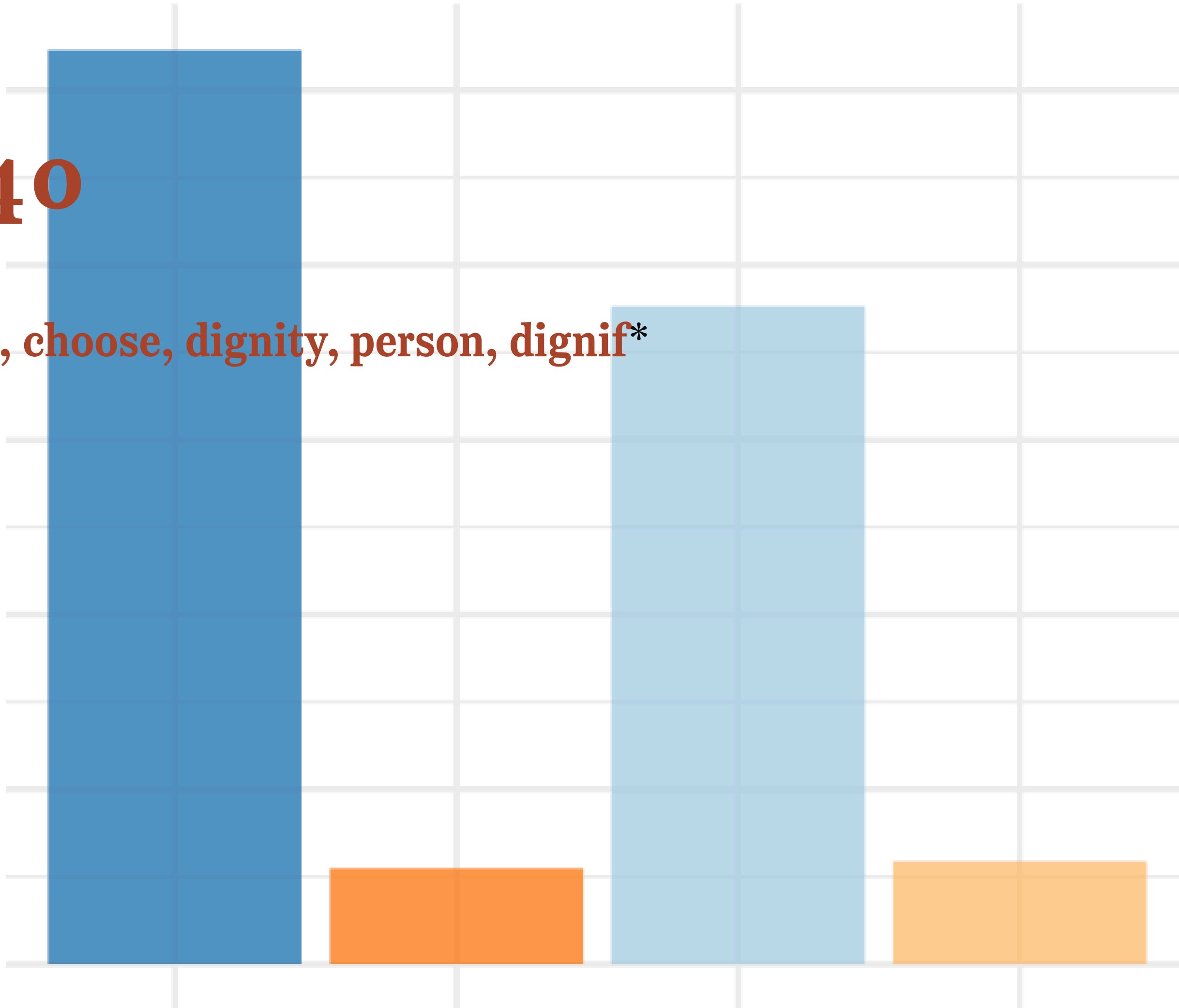
0.04

0.03

0.02

0.01

0.00



TOPIC 16

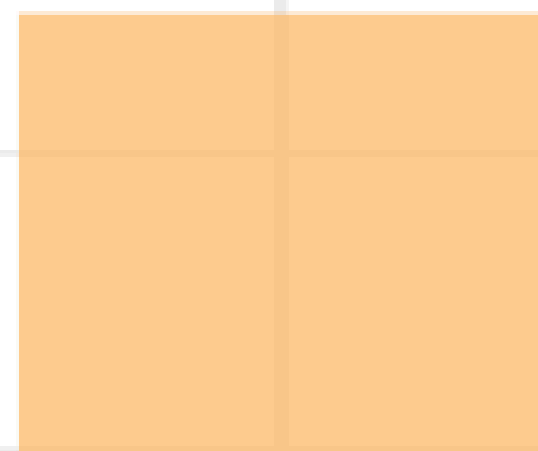
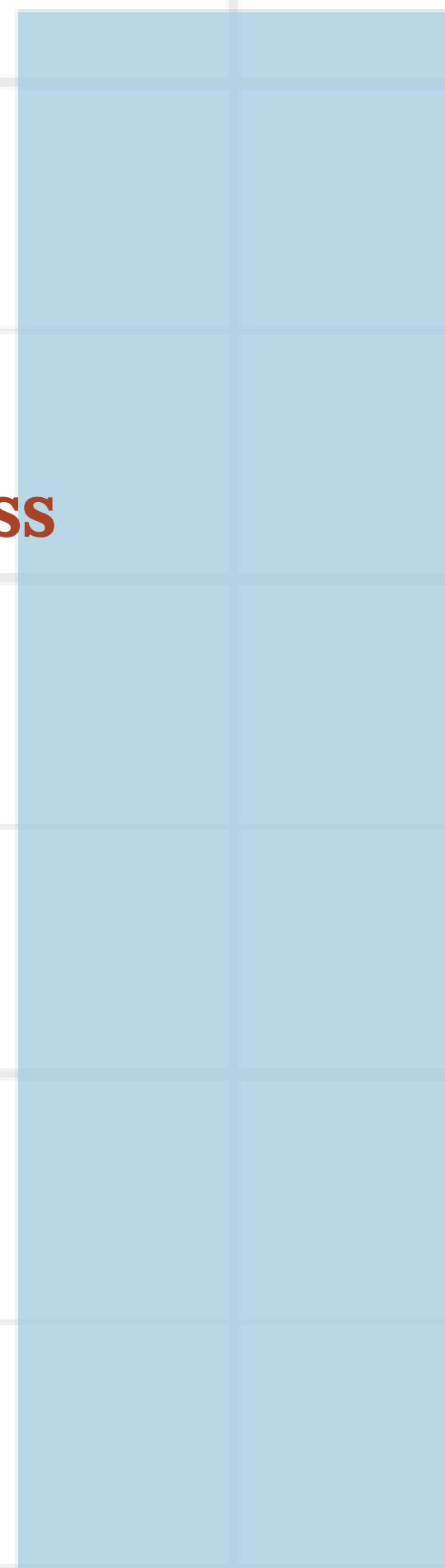
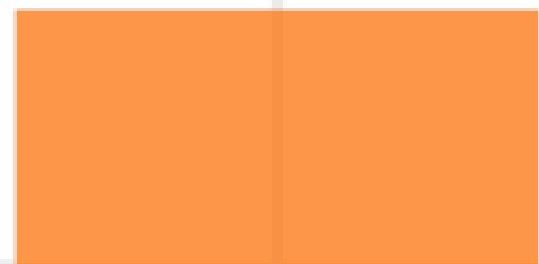
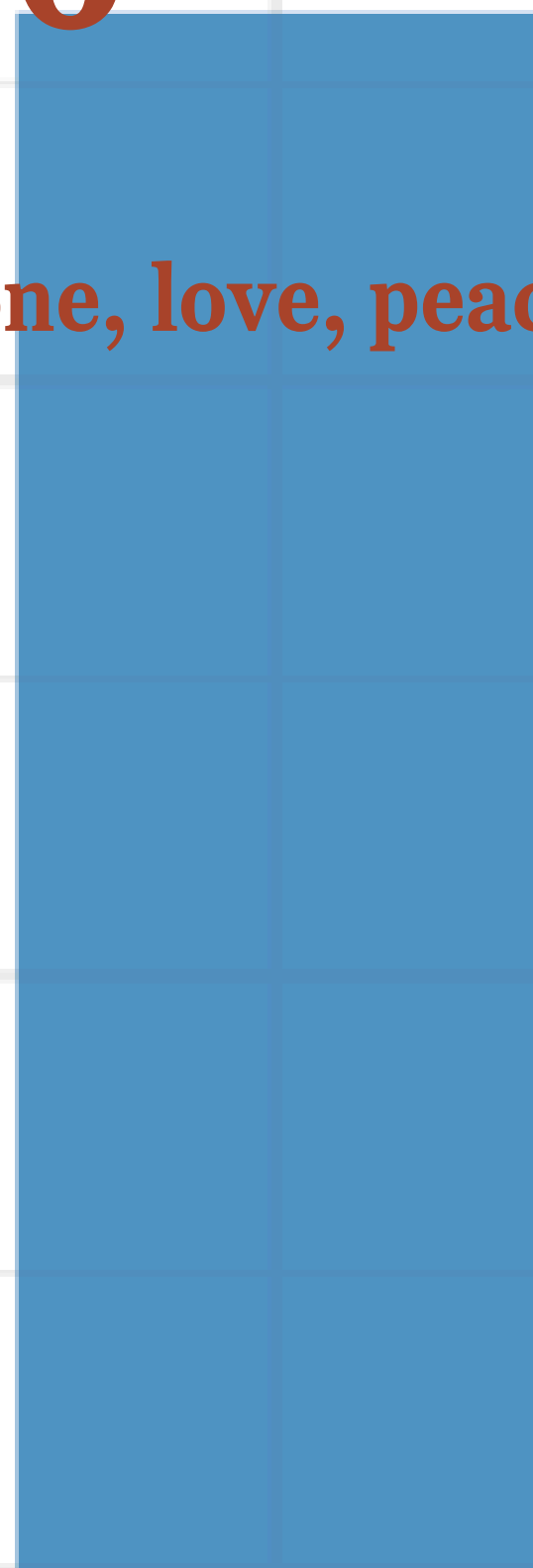
16: suffer, pain, one, love, peace, hope, distress

0.06

0.04

0.02

0.00



PRESS CORPORA

UK press articles which meet the following criteria:

- Available on LexisNexis.
- Between 23 March and 22 December 2021 or 7 December 2023 and 16 August 2024.
- Matching search terms “assisted suicide” OR “assisted dying” OR “euthanasia” OR “assisted to die” OR “assisted death” OR “assisted in dying” OR “assisted end to life” OR “death with dignity” OR “die with dignity” OR “die in dignity” OR “right to die” OR “end of life choice” OR “end of life assistance”

TERMINOLOGY: “ASSISTED DYING”

- Term used in current bill’s title
- Prevalent in consultations and media
- Primarily used by supporters
- Distinguished from suicide by supporters of the term as “people in question are not choosing death over a healthy life”
- Relatively new term (last 15 years in UK)
- Can emphasise disease/death “process” underway

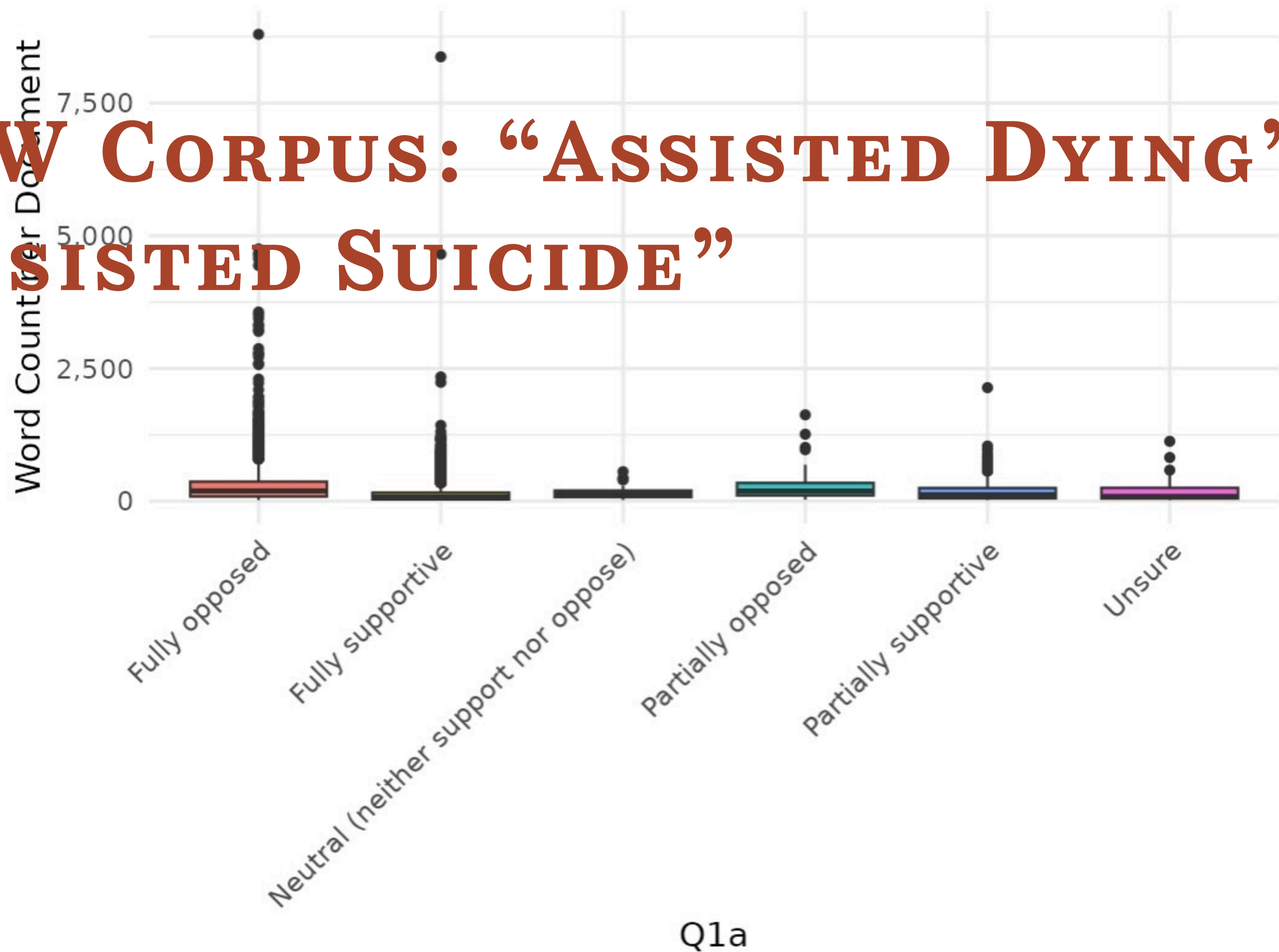
TERMINOLOGY: “ASSISTED SUICIDE”

- Primarily used by opponents
- Argued to be more legally/practically accurate
- Critics say “assisted dying” is a euphemism
- Retains the moral/religious stigma of “suicide”
- Can emphasise individual’s action

TERMINOLOGY: “EUTHANASIA”

- Greek “good death”
- Not technically accurate for current bill as euthanasia is physician-administered and this bill requires self-administration

NOW CORPUS: “ASSISTED DYING” vs “ASSISTED SUICIDE”



‘UP YOURS AT THE END OF LIFE’

*Opposition, Emotion, and Overlap in a
Corpus of Scottish Debates on Assisted Dying*

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Slides:

LEVERHULME
TRUST _____

